

CENTRO DE SALUD CONDUCTUAL DEL SUR

Behavioral Health Progress Note · Confidential — HIPAA Protected
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PATIENT	DATE OF BIRTH	MRN
Demo, Patient A.	03/14/1985 (Age 41)	PR-2024-00847
DATE OF SERVICE	VISIT TYPE	PLACE OF SERVICE
06/20/2026	Telehealth (Video)	02 — Telehealth
RENDERING PROVIDER	PROVIDER NPI	PAYER / PLAN
Dr. María E. González, MD	1457823690	ASES / Mi Salud — Plan Vital
AUTHORIZATION #	TELEHEALTH CONSENT	SESSION LENGTH
NOT ON FILE — PENDING	Signed on file: 04/10/2026	60 minutes
DIAGNOSES		

#	ICD-10-CM CODE	DESCRIPTION	TYPE
1	F32.1	Major depressive disorder, single episode, moderate	Primary
2	F41.1	Generalized anxiety disorder	Secondary
S — SUBJECTIVE			

Patient presents via secure video telehealth platform (Doxy.me) for follow-up individual psychotherapy. Patient reports persistent low mood over the past two weeks, difficulty concentrating at work, and early morning awakening approximately 3-4 times per week. Denies suicidal ideation, homicidal ideation, or active hallucinations. Reports mild improvement in anxiety symptoms since last session.

Patient states: "I feel a little better about the anxiety but the sadness is still there in the mornings." Social support remains limited; patient lives alone and reports minimal contact with family in Ponce.

O — OBJECTIVE

Mental Status Exam: Alert and oriented x4. Appearance appropriate; grooming adequate via video.
Speech: normal rate and rhythm. Mood: "sad." Affect: constricted, somewhat reactive. Thought process: linear and goal-directed. Thought content: no delusions. Perception: denies AH/VH. Cognition: intact to interview. Insight: fair. Judgment: fair. Impulse control: adequate.

PHQ-9 Score: 14 — Moderate depression (documented in chart)

GAD-7 Score: 9 — Mild-moderate anxiety (documented in chart)

A — ASSESSMENT

Patient continues to meet diagnostic criteria for Major Depressive Disorder, Single Episode, Moderate (F32.1) and Generalized Anxiety Disorder (F41.1). Partial response to psychotherapy. PHQ-9 improved from 17 at intake to 14 today. GAD-7 stable. Ongoing psychosocial stressors (employment, social isolation) contributing to symptom persistence.

P — PLAN

- Continue weekly individual psychotherapy using Cognitive Behavioral Therapy (CBT) techniques, focusing on behavioral activation and worry management protocols.
- Medication management review conducted today: current SSRI (Escitalopram 10 mg QD) dosage adequate; no changes made. Patient tolerating medication well with no reported side effects.
- Referral to group therapy discussed — patient receptive, will consider for next cycle.
- Safety plan reviewed; patient has active safety plan on file. No acute risk identified.
- Return for follow-up in one week. Authorization renewal to be initiated by billing office prior to next visit.

Time spent in psychotherapy: 60 minutes (55 min face-to-face + 5 min documentation).

More than 50% of encounter dedicated to psychotherapy — supports 90837 billing.

Medical decision-making (E&M):

Moderate complexity. Two chronic conditions reviewed, medication reconciliation performed, one prescription reviewed. Service is independently documented and medically necessary — separate and distinct from the psychotherapy service above. Modifier 25 applies.